



How Data Drives Healthcare & Pharma

Guest lecture

May 4, 2026

Martin Černý, Principal, Real World Evidence CEE & CIS

Barbora Jakubová, Consultant, CZ&SK Consulting & RWE

Let us introduce ourselves

IQVIA introduction



Martin Černý

*Principal,
Real World Evidence CEE&CIS*

Martin.Cerny@IQVIA.com



Barbora Jakubová

*Consultant,
CZ&SK Consulting & RWE*

Barbora.Jakubova2@IQVIA.com



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- **PHARMA & HEALTHCARE DATA LANDSCAPE**
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IQVIA

OUR VISION

**Power smarter healthcare for
everyone, everywhere**

Invest in **domain expertise** to gain the intelligence required to operate and transform essential processes

88,000+

Experts serving clients in **100+** countries

12,000+

Technology experts

4,000+

Advanced analytics / data scientists / statisticians

3,700+

Epidemiologists / RWI experts



2,500+

PhDs

2,200+

Medical doctors

7,500+

Services experts

16,800+

Off-shore delivery support

Apply advanced data science technologies
to **unparalleled data resources** to generate
actionable insights

1.2B+
Non-identified
patient records

85%+
Global pharma
sales tracked

4.9M
Investigators

64+
Petabytes of
unique data



Global &
local data

Data science
proficiency

~1M
Data feeds

~25M
Healthcare
professionals

150,000+
Data suppliers



Patients



Physicians



Hospitals



Researchers



Pharma



Biomarkers
and genomics



Disease and
treatment



Labs and
diagnostics

Deliver integrated capabilities and dedicated teams to support Pharma stakeholders at every stage



- Asset & Portfolio Strategy
- Value & Access
- Patient Centricity
- Brand & Commercial Strategy
- M&A (Pharma, Biotech, Financial Institutions)
- Digital Health Strategy
- AI/ML powered Strategy
- Competitive Intelligence

- Commercial Effectiveness Services
- Brand & Integrated Research Solutions
- Technology Consulting
- Payer Evidence Solutions
- Patient Centered Endpoint Solutions
- Real World Consulting
- R&D Consulting

- GMI
- Information Offerings
- Orchestrated Customer Engagement
- Performance management
- Integrated Compliance Solutions
- Smart Platform (AI/ML offering)
- Real World Analytics & Databases (e.g., E360)
- Patient-level Data

- HCP engagement strategy
- Contract sales organization
- Key Account Management
- Performance Measurement & Management
- Patient and medical Communications
- Medical Affairs

- Clinical Development
- Strategic Drug Development
- Drug Discovery
- Regulatory Affairs

- Pharmacovigilance
- Real World late phase studies
- Connected Devices (i.e., Apple collab)

PRESS RELEASE

IQVIA Unveils IQVIA.ai, a Unified Agentic AI Platform Powered by NVIDIA to Improve Efficiency and Decision Making Across Life Sciences

Mar 16, 2026

[Home](#) / [Newsroom](#) / [IQVIA Unveils IQVIA.ai, a Unified Agentic AI Platform Powered by NVIDIA to Improve Efficiency and Decision Making Across Life Sciences](#)

RESEARCH TRIANGLE PARK, NC – March 16, 2026 – IQVIA Holdings Inc. (NYSE:IQV), a leading global provider of clinical research services, commercial insights and healthcare intelligence, today announced at [NVIDIA GTC](#) the launch of [IQVIA.ai](#), a unified agentic AI platform purpose-built to help life sciences organizations transform how they operate, make decisions and innovate across clinical, commercial and real-world domains.

Combining IQVIA's deep life sciences expertise, Healthcare-grade AI® and data assets with NVIDIA Nemotron, NeMo Agent Toolkit, Dynamo and LangChain, IQVIA.ai is designed to continuously learn from complex operational feedback, while operating securely and in alignment with healthcare

IQVIA Literature AI Platform

Cut literature review times by over 75% while improving the quality and consistency of outputs with the IQVIA Literature AI Platform. Its architecture is underpinned by proprietary, domain-tuned agents that deliver expert-level performance that cannot be replicated by generic models.

Available: Now

Med-R1 Deep Research Agent

IQVIA Medical Reasoning (Med-R1 8B) set a new standard for interpreting complex data in regulated healthcare environments. Now, Med-R1 Deep Research Agent goes further by synthesizing evidence across healthcare, regulatory and biomedical sources to deliver citation-grounded answers in response to complex clinical or biomedical questions.

Available: Now

GMI Agent

IQVIA.ai on 7 global IQVIA data sources, including MIDAS, ChannelDynamics, Market Prognosis, answers complex business questions with deeper insights and recommendations.

Available: Now



IQVIA – Global #1 Leadership Areas

1. #1 in global healthcare data assets
2. #1 in pharmaceutical commercial intelligence
3. #1 in real-world evidence (RWE) at scale
4. #1 in clinical trials
5. #1 in healthcare and life sciences consulting
6. #1 in AI-powered solutions for pharma and healthcare

Claude

🧠 Domain Knowledge vs. Python Skills in the AI Era — Which Matters More?

Short answer: Both. And together they are unstoppable.

🔬 Why domain knowledge is the foundation

- AI can write code — but it cannot replace **knowing what question to ask**
- In pharma and healthcare, **wrong assumptions kill studies** — no syntax fixes a flawed hypothesis
- Clients and regulators pay for **judgment and interpretation**, not code execution

💻 Why Python still matters

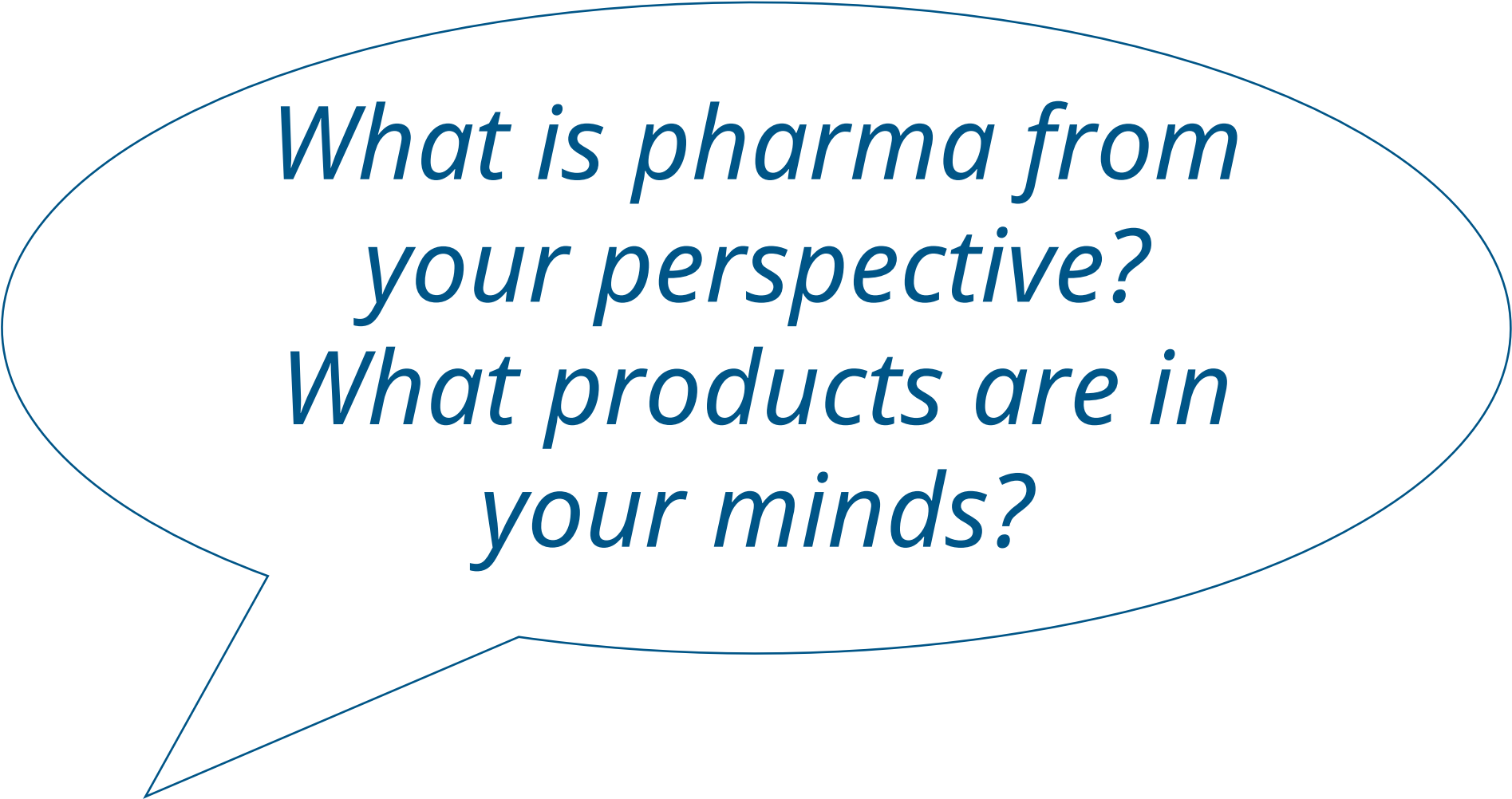
- Speeds up analysis and reduces dependency on others
- Lets you **validate, direct, and challenge** technical outputs
- Gives you credibility with data scientists and statisticians
- Increasingly expected at senior level — not deep coding, but enough to **read and reason**

🏆 The winning combination in 2026

Profile	Value
Python only, no domain knowledge	🟡 Increasingly replaceable by AI
Domain only, no tech skills	🟢 Strong, but needs technical partners
Domain knowledge + Python + AI fluency	🏆 Highest value, hardest to replace



Pharma industry



*What is pharma from
your perspective?
What products are in
your minds?*

Drugs can be divided into various segments – each having different patient base, regulation, marketing & business strategy

Drug classification

These classes are not mutually exclusive (e.g., prescription biologics)

**Prescription
Drugs (Rx)**



**Orphan
Drugs**



Generics



**Over the Counter (OTC) Drugs
& Consumer Health (CH)**



Biologics



Biosimilars



The Rx market reached ~ CZK 112 billion in 2025

Example of Rx brands

KEYTRUDA®
(pembrolizumab) Injection 100 mg


mounjaro™
(tirzepatide) injection

OPDIVO™
(nivolumab)

OCREVUS®
ocrelizumab 300MG/10ML
INJECTION FOR IV

OZEMPIC®
semaglutide injection

 **detralex®**


kaftrio

 **DARZALEX®**
(daratumumab)
injection for intravenous infusion
100 mg/5 mL, 400 mg/20 mL

HEMLIBRA®
emicizumab

The Consumer Health (CH) market accounted for ~ CZK 35 billion in 2025

Example of CH brands



We help most of the major pharma and biotech companies as well as other stakeholders in pharma industry

Example of our customers

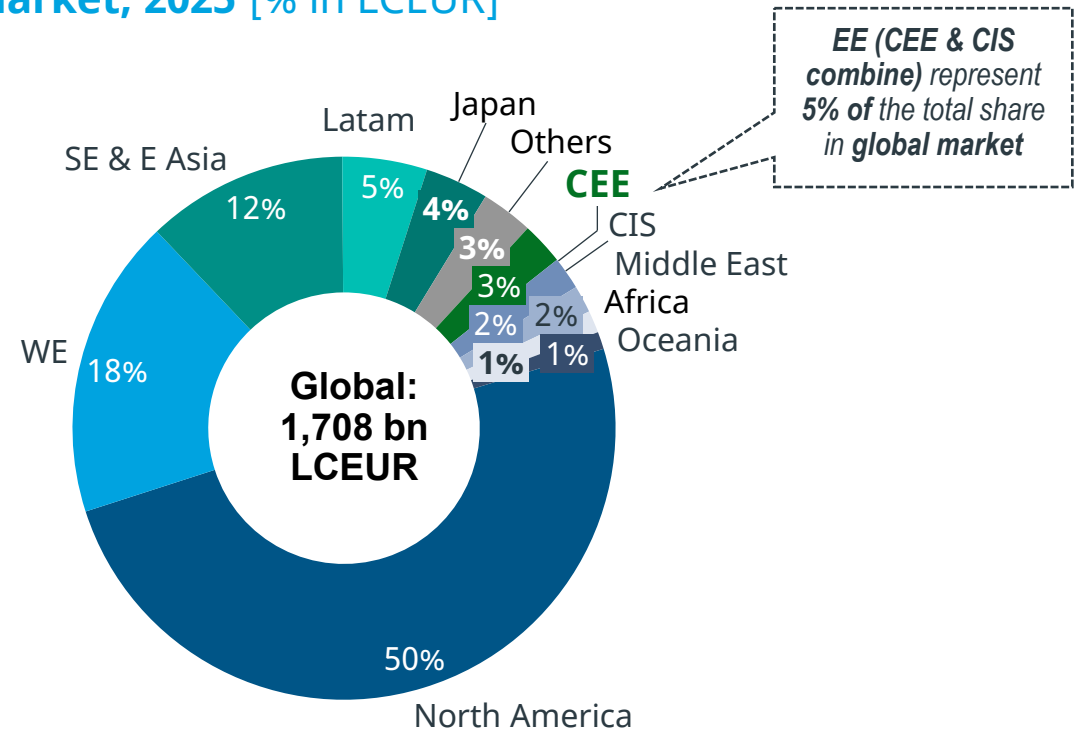


CEE makes up nearly 3% of the global pharma market and is expected to see the fastest growth in the next 5 years

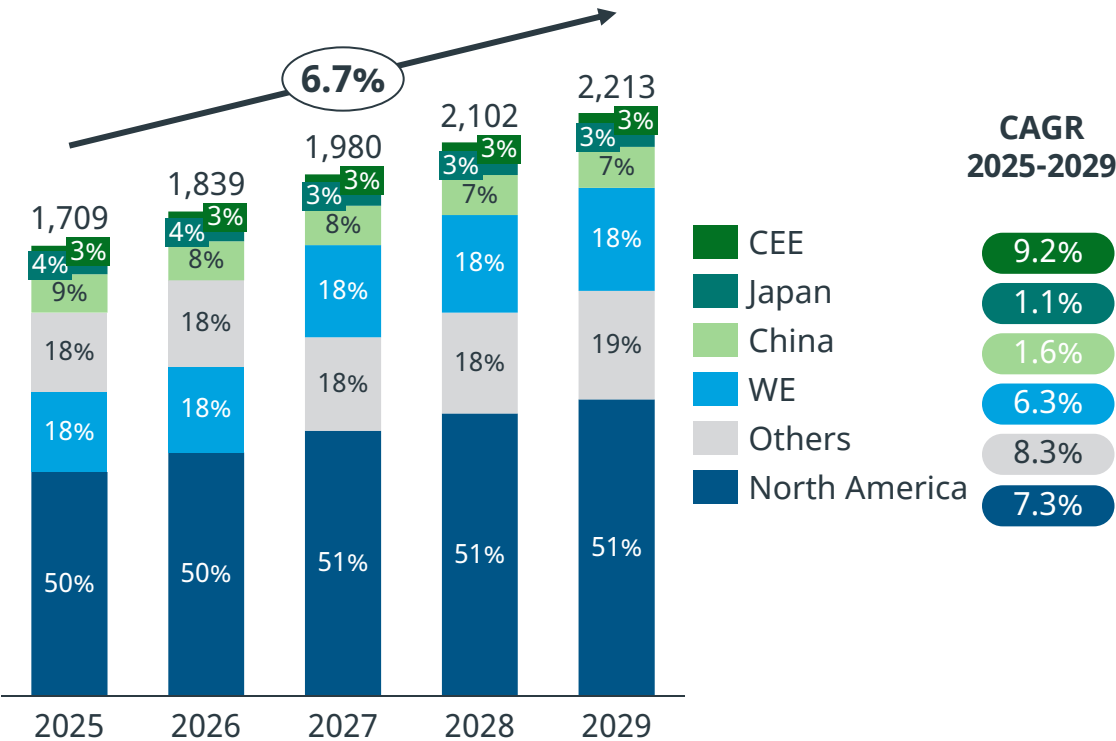
Pharma market overview [Rx + regOTC]

Global view

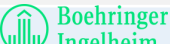
Size of pharma markets and their share in global market, 2025 [% in LCEUR]



Global pharma markets value and YoY growth of key regions, 2025-2029 [bn LCEUR]

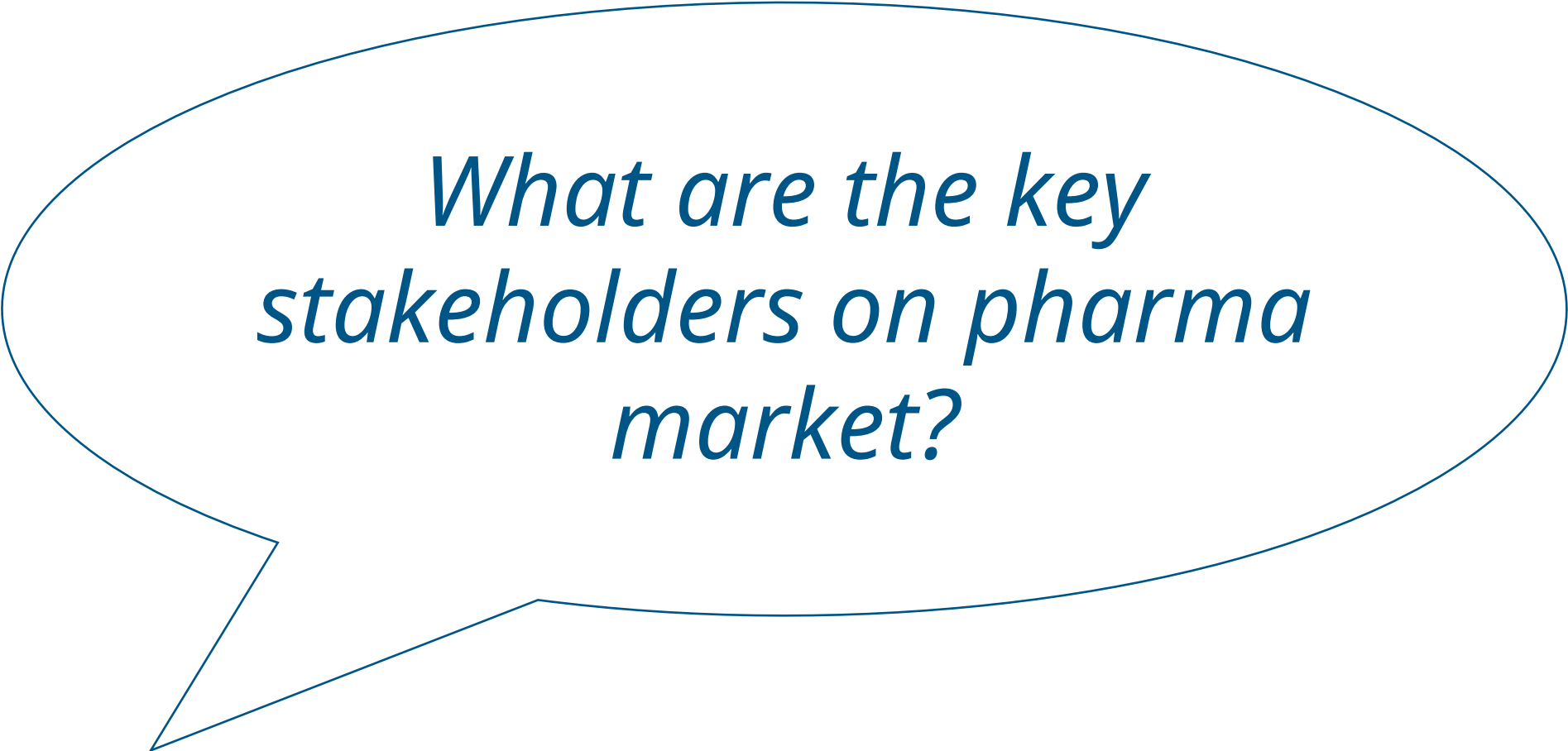


CEE: Poland, Romania, Hungary, Czech Republic, Slovakia, Croatia, Bulgaria, Slovenia, Lithuania, Serbia, Latvia, Bosnia, Estonia, Albania, Kosovo, Montenegro, Macedonia
CIS: Russia, Kazakhstan, Uzbekistan, Mongolia, Moldova, Armenia, Azerbaijan, Belarus, Georgia, Kyrgyzstan, Tajikistan
WE: Germany, France, UK, Italy, Spain, Netherlands, Belgium, Greece, Portugal, Sweden, Austria, Switzerland, Finland, Norway, Ireland
Source: IQVIA Local Databases, MIDAS, Pharma market includes Retail and Hospital sales; IQVIA Market Prognosis Global 2025-2029
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www.FiercePharma.com The top 20 pharma companies by 2024 revenue					
	Company	2024 revenue [\$ bn]	2023 revenue [\$ bn]	PPG growth [%]	Comments (2024 vs. 2023)
1		89	85	4%	Stelara LoE (-5%), diversification through Tremfya, Rybrevant, and medtech (35%!)
2		65	65	3%	Vabysmo (\$4.5B) replaces Tecentriq in top 3; Hemlibra (+12%), diagnostics (25%!)
3		64	60	7%	46% of revenue from Keytruda (+18%) & Gardasil (-3%); IRA pricing and LoE risks, animal (10%!)
4		64	60	7%	Recovery from -41% in 2023; Seagen acquisition (+\$3.4B)
5		56	54	4%	Humira -38% (LoE) offset by Skyrizi & Rinvoq (\$17B, both +50%)
6		54	46	18%	Growth drivers: Forxiga (+29%), Enhertu, Beyfortus; China sales decline in Q4
7		50	45	12%	Growth drivers: Entresto (+30%), Cosentyx, Kisqali (+46%), Scemblix & Fabhalta approvals
8		48	45	7%	Eliquis (\$13.3B, +9%) Revlimid decline; Growth portfolio (+17%, Opdivo, Reblozyl)
9		45	34	32%	Growth drivers: Mounjaro, Zepbound; \$27B U.S. plant expansion
10		42	34	26%	Growth drivers: Ozempic (\$17B) & Wegovy (\$8B) Shortages lifted; \$9B plant expansion
11		41	38	9%	Growth drivers: new launch Altuviio & Rezurock; Beyfortus (\$1.84B)
12		40	38	3%	HIV (+13%) offsets Arexvy RSV vaccine drop (-70%); Blenrep regains relevance
13		33	28	19%	Horizon acquisition adds Tepezza (\$1.85B); Biosimilar launches key to strategy
14		31	27	10%	Growth driver: Entyvio (+11%); IBD and rare disease growth; Vyvanse LoE impact softened
15		29	28	5%	Growth drivers: Jardiance (+15%) & Ofev (+9%) + Investing in 25 launches by 2030
16		29	27	6%	Growth driver: HIV Biktarvy (\$13.4B); Lenacapavir PrEP TBL in 2025; Oncology (Trodelvy)
17		26	26	0%	Xarelto (-15%) & Eylea LoE; Pressure from litigation, cost cuts; Nubeqa & Kerendia rising
18		18	17	2%	Recovery from -5%; blockbusters (Erbixux, Glucophage, Mavenclad) all +10%; PS decline
19		17	16	4%	Growth Drivers: Austedo (+36%), Ajovy (+18%) & stable generics (+11%)
20		15	14	7%	Privigen & Hizentra (+15%), hemophilia & plasma strong, Vifor stabilizing, Vaccine (-9%)



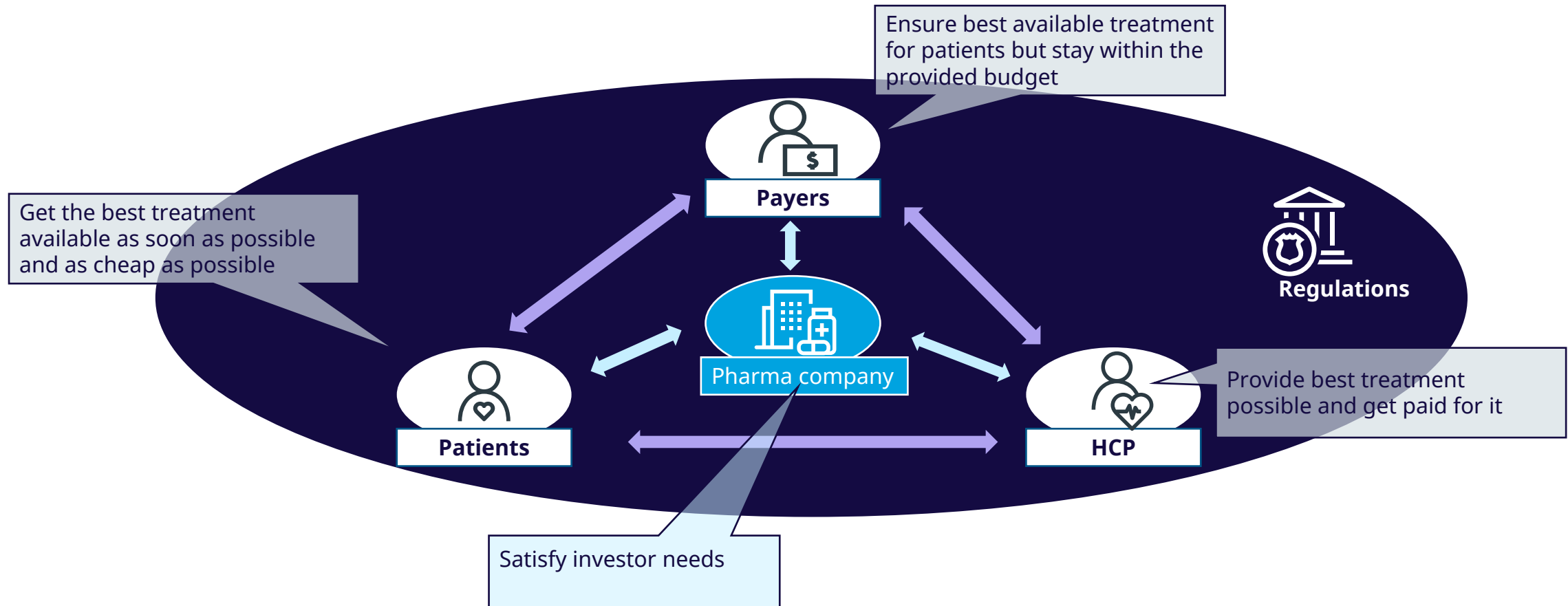
Key stakeholders in pharma / healthcare



*What are the key
stakeholders on pharma
market?*

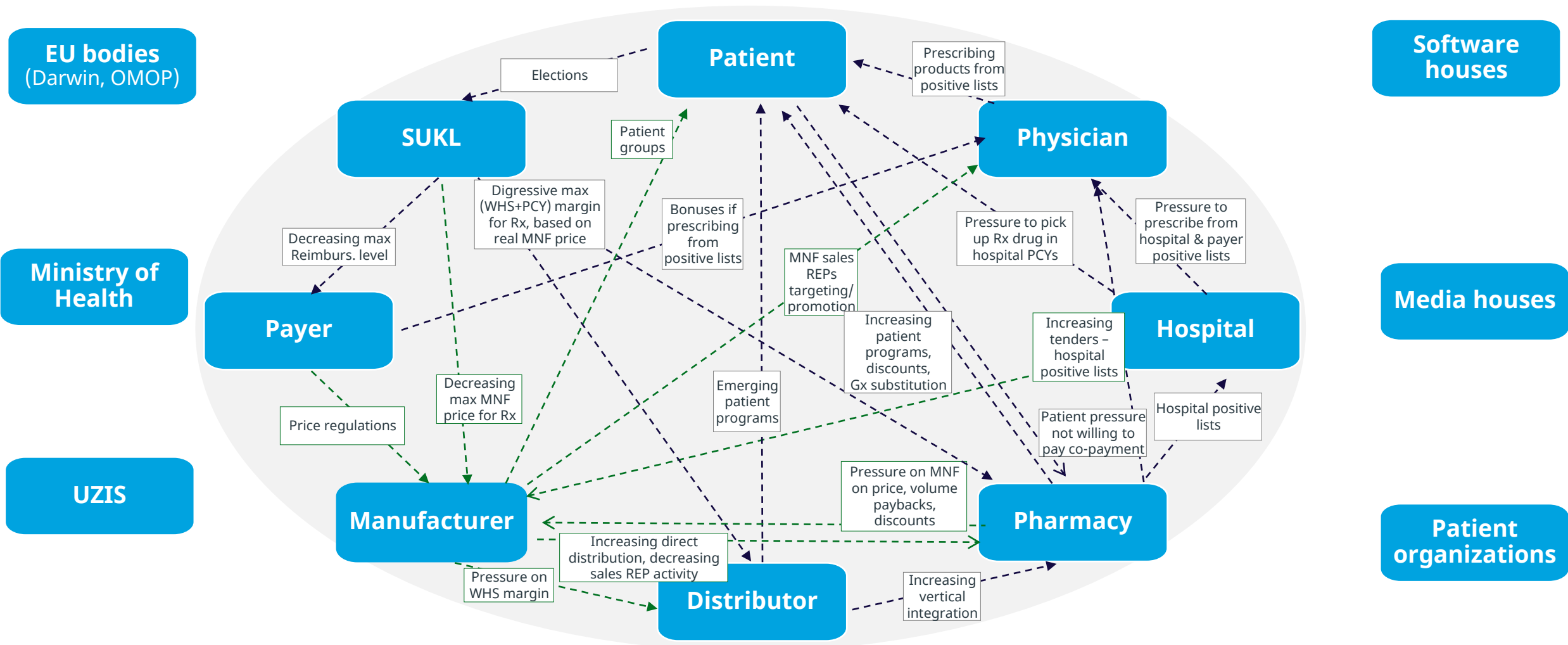
When operating, pharma companies have to manage the relations with several stakeholders – each with different priorities...

Key players on the market: Simple view



... in reality the situation is much more complex

Key players on the market: Real complex situation

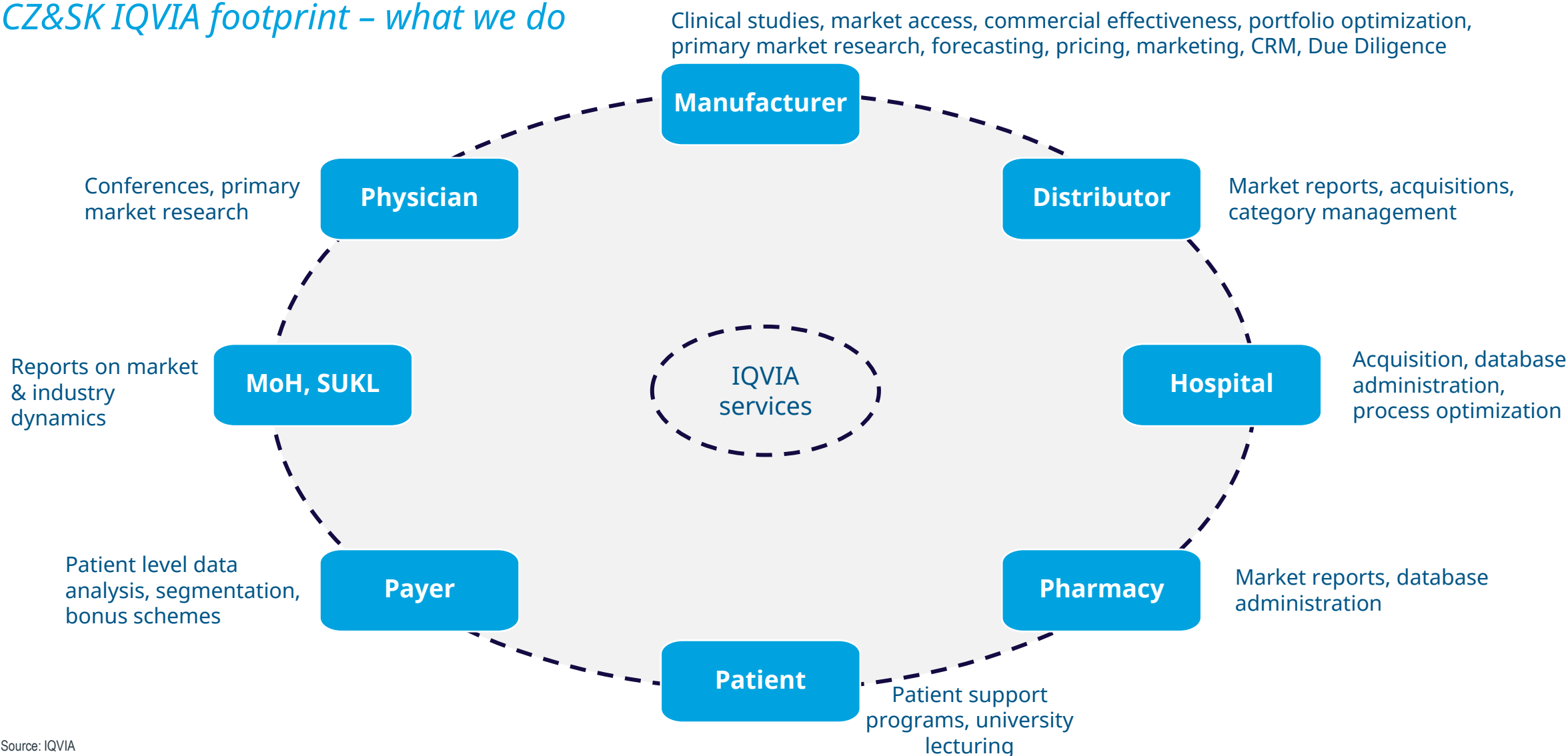


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IQVIA CZ&SK is supporting all key local stakeholders in pharma/ healthcare industry...

CZ&SK IQVIA footprint – what we do

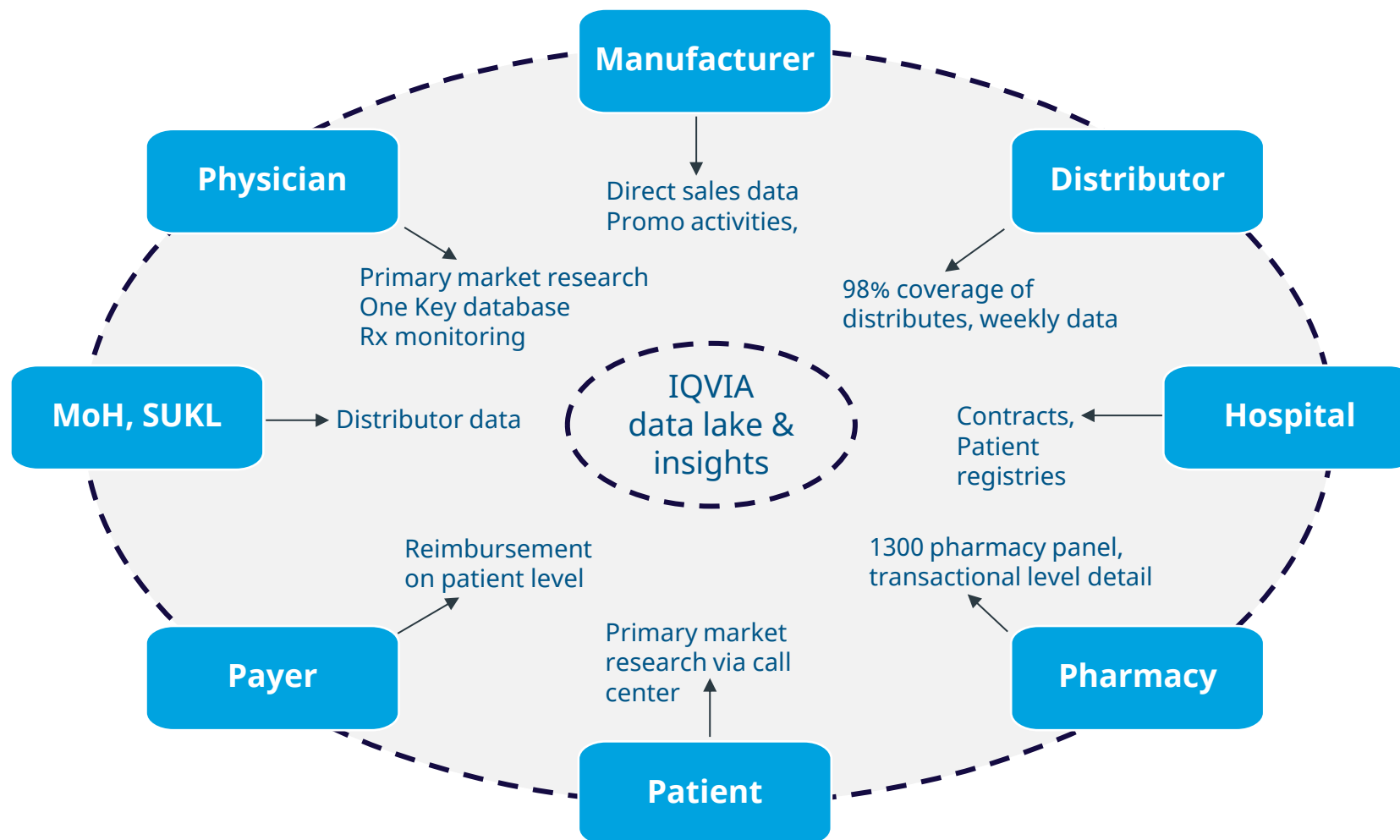


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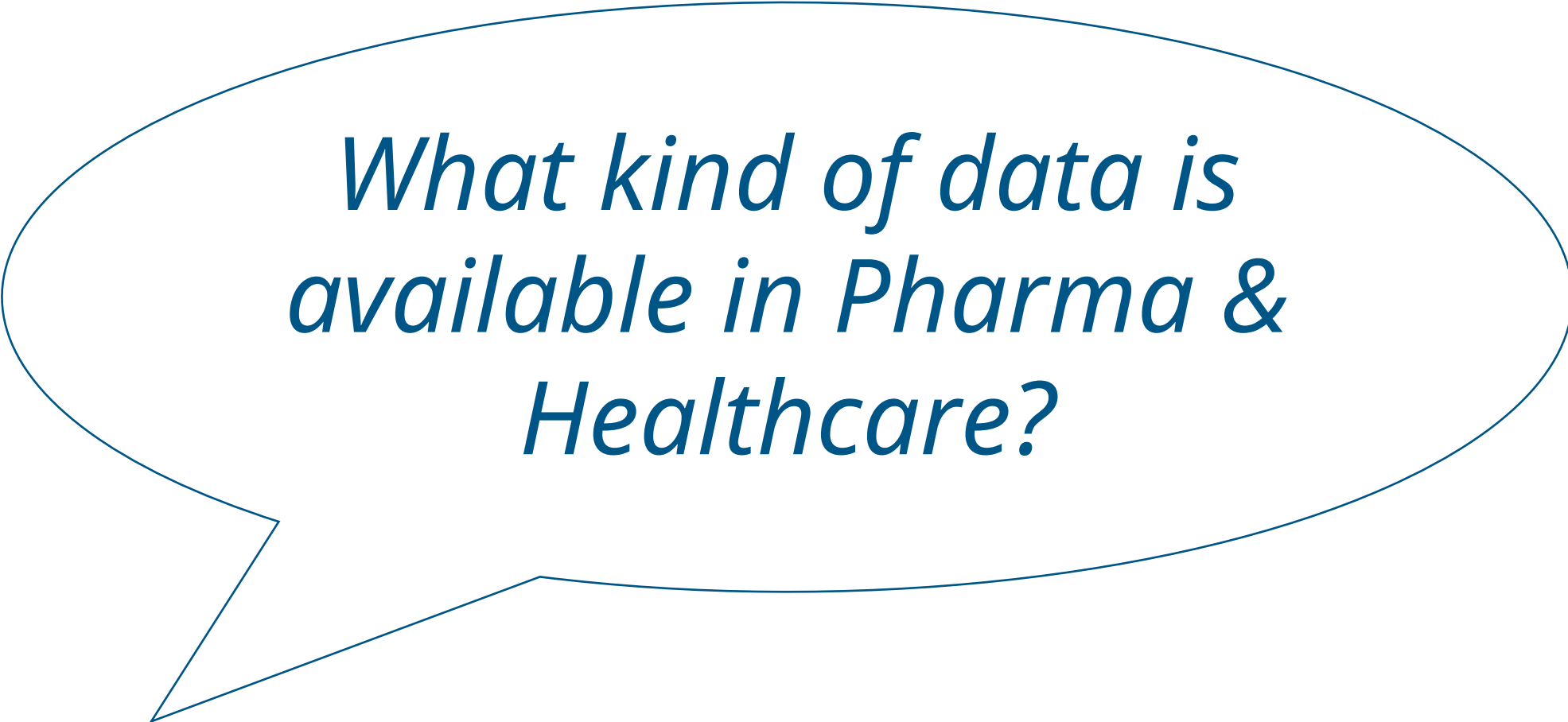
... via consolidating various data sources and deriving insights

CZ&SK IQVIA footprint – data assets





Pharma & Healthcare Data Landscape



*What kind of data is
available in Pharma &
Healthcare?*

Data from distributors or pharmacies are typical data that Pharmacos use for analyses of market trends

Simplest data in pharma are the sales data – from national level to an “EET style” transactions

Pharmacy name	Transaction id	Time/date	Product id	# of packs	Price/ copayment	Reimbursement

An abstract graphic on the left side of the slide, featuring flowing, wavy lines in shades of blue and teal, interspersed with numerous small, glowing blue dots, creating a sense of data movement and connectivity.

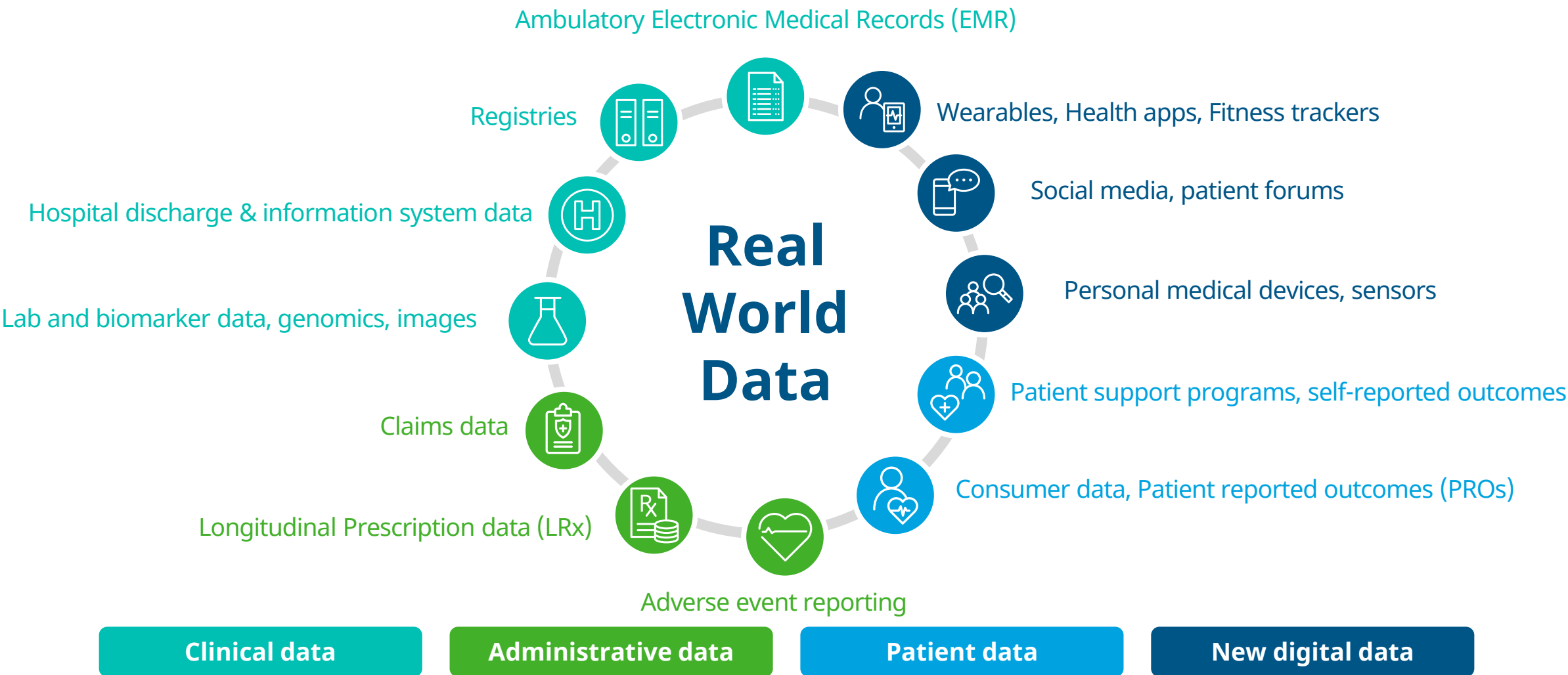
Real World Data
Real World Evidence



*What health data is
available about you,
the patient?*

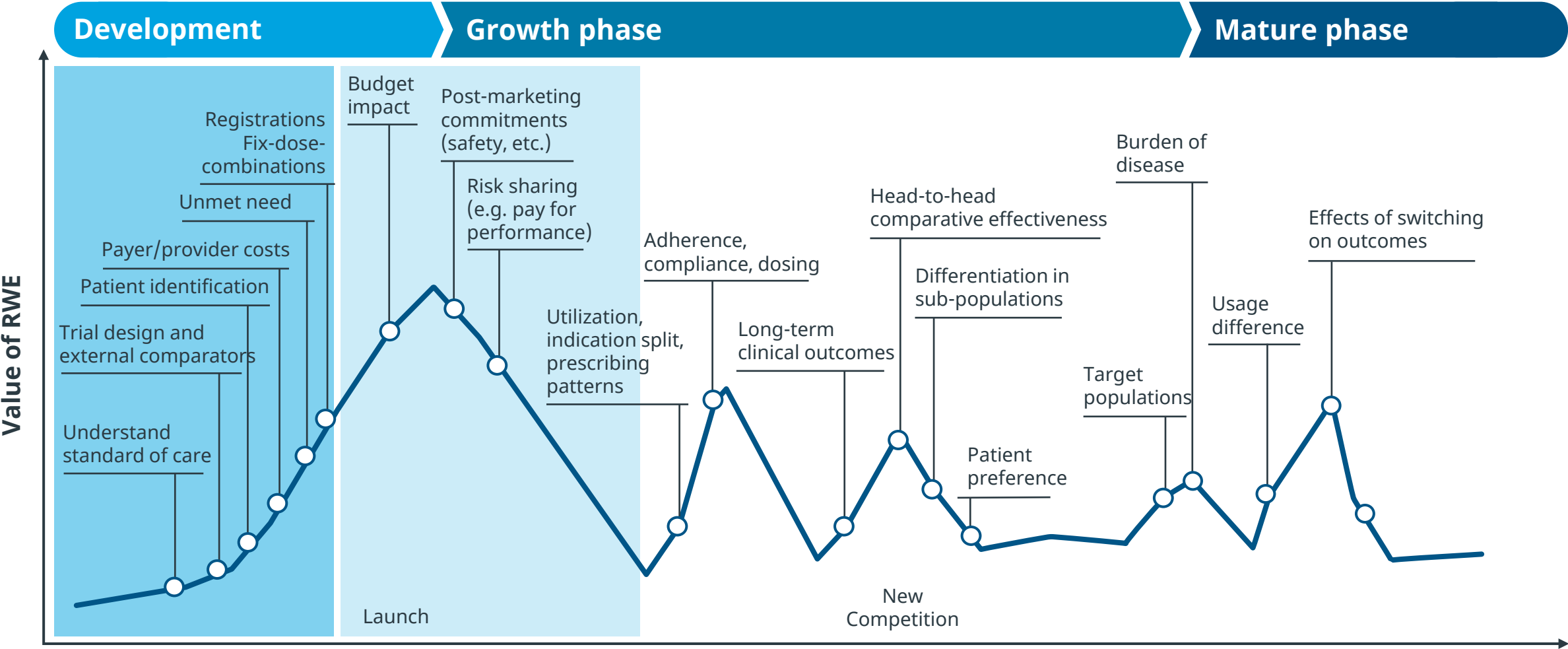
Real World Data come from a variety of sources with varying granularity and use cases and country availability

What type Real World data exist?



RWE creates value across the product lifecycle, with the greatest impact when used early to shape decisions

When RWE creates the greatest strategic value?



Typically, we distinguish between 2 RWD types: primary & secondary – both have its unique advantages in RWE

Primary vs Secondary Data



Primary Data

Data collected from first-hand sources for the specific purpose of the study

Examples include:

- **Patient Reported Outcomes**
- **Case Report Forms**

Primary data is collected prospectively, in systems that are set-up for the study.



Secondary Data

Data from existing sources collected during routine practice

Examples include:

- **Claims**
- **Medical records**
- **Existing registries**

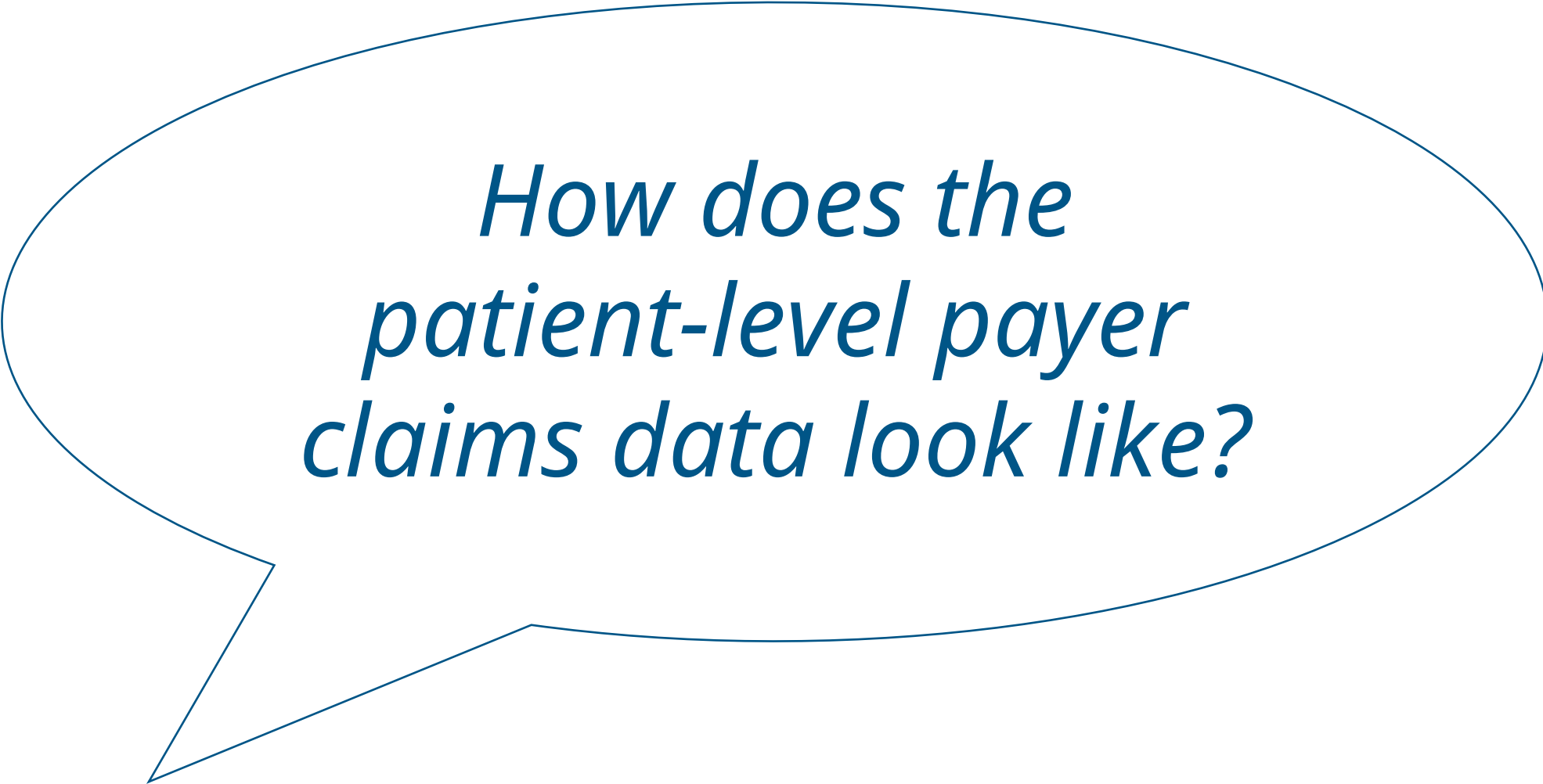
There are many modalities of collecting secondary data, using different set-ups.

Various types of Real-World-Data are available across Eastern Europe, site-based collection possible across all countries

Accessible Real World Data type availability by country (1/2)

Legend: ✓ Available | ✓ Limited availability | ✗ Not available

Real World data	Typical use case	Poland 	Czechia 	Slovakia 	Hungary 	Romania 	Bulgaria 
Drug consumption linked to patient ID	# of patients on drug, persistency, line of therapy, switch, indication split	✓ (IQVIA panel, 55% retail coverage)	✓ (claims, 100% coverage, UZIS)	✓ (claims, 100%, until 2020, NCZI)	✓ (claims, 100% coverage, NEAK)	✗	✓ (IQVIA panel, 30% retail coverage)
Procedures and test linked to patient ID	Patient journey, mortality, hospitalizations, testing, burden of disease	✗	✓ (claims, 100% coverage)	✗	✓ (claims, 100% coverage)	✗	✓ (EMR at GPs only, 50% coverage)
Lab values linked to patient ID and drug consumption	Drug effectiveness	✓ (IQVIA panel, till 2024, 50% coverage)	✓ (to be available in 2026/2027)	✗	✗	✗	✓ (EMR at GPs only, 50% coverage)
Disease specific registries with detailed outcome data	Drug effectiveness, burden of disease	✓ (limited outcome data)	✓ (IQVIA run, state run, 3 rd party)	✗	✓ (limited outcome data)	✓ (IQVIA & state run, now being set-up)	✓ (limited outcome data)
Cross sectional Oncology data (IQVIA Oncology Dynamics)	# of patients, disease dynamics	✓	✓ (till 2023)	✓ (till 2023)	✗	✓ (till 2024)	✓ (till 2024)
EMR data in outpatient settings	Drug effectiveness	✗	✗	✗	✗	✗	✓ (GPs, 50% coverage)
Hospital HIS data in inpatient setting	Drug effectiveness	✓ (ad hoc at selected sites)	✓ (ad hoc at selected sites) + OEN	✗	✓ (ad hoc at selected sites)	✓ Selected private providers	✓ (ad hoc at selected sites)
Chart reviews / prospective site based data collection / PROs	Drug effectiveness, safety, burden of disease, PROs	Yes, available across all countries					



*How does the
patient-level payer
claims data look like?*

Example of patient-level data

File name:
Breast cancer

Number of rows:
2 million

Number of columns: 17

Csv file size: 225 MB

pacient_id	mesic	pacient_transakce_cislo	rok	sukl_kod	atc_kod	nazev_lp	doplnek_lp	sila_lp	denni_davka_baleni	denni_davka_jednotka	odbornost	odbornost_prevazujici	diagnoza	vek_transakce	vek_25	suma_mnozstvi	
13	2024-10		23	2024	0253468	L02BG03	ARIMIDEX	1MG TBL FLM 28	1MG	28.0000	MG				50-64	50-64	6.000
14	2022-08		1	2022	0084229	L01AA01	ENDOXAN	200MG INJ/INF PLV SOL 10X1	200MG	43.1599	MG	402	402	C504	50-64	65+	0.100
14	2022-08		1	2022	0084231	L01AA01	ENDOXAN	1G INJ/INF PLV SOL 1	1G	21.5800	MG	402	402	C504	50-64	65+	1.000
14	2022-08		1	2022	0239780	L01DB03	EPIDUBICIN ACCORD	2MG/ML INJ/INF SOL 1X25ML	2MG/ML	8.0924	MG	402	402	C504	50-64	65+	3.000
14	2022-09		2	2022	0084231	L01AA01	ENDOXAN	1G INJ/INF PLV SOL 1	1G	21.5800	MG	402	402	C504	50-64	65+	1.000
14	2022-09		2	2022	0084229	L01AA01	ENDOXAN	200MG INJ/INF PLV SOL 10X1	200MG	43.1599	MG	402	402	C504	50-64	65+	0.100
14	2022-09		2	2022	0239780	L01DB03	EPIDUBICIN ACCORD	2MG/ML INJ/INF SOL 1X25ML	2MG/ML	8.0924	MG	402	402	C504	50-64	65+	3.000
14	2022-09		3	2022	0194294	L03AA14	LONQUEX	6MG INJ SOL ISP 1X0,6ML I	6MG	20.0000	MG		402	C504	50-64	65+	1.000
14	2022-09		3	2022	0239780	L01DB03	EPIDUBICIN ACCORD	2MG/ML INJ/INF SOL 1X25ML	2MG/ML	8.0924	MG	402	402	C504	50-64	65+	2.000
14	2022-09		3	2022	0084231	L01AA01	ENDOXAN	1G INJ/INF PLV SOL 1	1G	21.5800	MG	402	402	C504	50-64	65+	1.000
14	2022-10		4	2022	0239780	L01DB03	EPIDUBICIN ACCORD	2MG/ML INJ/INF SOL 1X25ML	2MG/ML	8.0924	MG	402	402	C504	50-64	65+	2.000
14	2022-10		4	2022	0084231	L01AA01	ENDOXAN	1G INJ/INF PLV SOL 1	1G	21.5800	MG	402	402	C504	50-64	65+	1.000
14	2022-11		5	2022	0250270	L01FY01	PHESGO	1200MG/600MG INJ SOL 1X15ML	1200MG/600MG	1.0000		402	402	C504	50-64	65+	1.000
14	2022-11		6	2022	0131863	L01CD01	PACITAXEL KABI	6MG/ML INF CNC SOL 1X50ML	6MG/ML	20.8092	MG	402	402	C504	50-64	65+	0.500
14	2022-11		7	2022	0131863	L01CD01	PACITAXEL KABI	6MG/ML INF CNC SOL 1X50ML	6MG/ML	20.8092	MG	402	402	C504	50-64	65+	0.500
14	2022-11		8	2022	0131863	L01CD01	PACITAXEL KABI	6MG/ML INF CNC SOL 1X50ML	6MG/ML	20.8092	MG	402	402	C504	50-64	65+	0.500
14	2022-11		9	2022	0250270	L01FY01	PHESGO	1200MG/600MG INJ SOL 1X15ML	1200MG/600MG	1.0000		402	402	C504	50-64	65+	1.000
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14	2022-12		11	2022	0250271	L01FY01	PHESGO	600MG/600MG INJ SOL 1X10ML	600MG/600MG	1.0000		402	402	C504	50-64	65+	1.000
14	2022-12		12	2022	0131863	L01CD01	PACITAXEL KABI	6MG/ML INF CNC SOL 1X50ML	6MG/ML	20.8092	MG	402	402	C504	50-64	65+	0.500
14	2022-12		13	2022	0131863	L01CD01	PACITAXEL KABI	6MG/ML INF CNC SOL 1X50ML	6MG/ML	20.8092	MG	402	402	C504	50-64	65+	0.500
14	2022-12		14	2022	0131863	L01CD01	PACITAXEL KABI	6MG/ML INF CNC SOL 1X50ML	6MG/ML	20.8092	MG	402	402	C504	50-64	65+	0.500
14	2022-12		15	2022	0250271	L01FY01	PHESGO	600MG/600MG INJ SOL 1X10ML	600MG/600MG	1.0000		402	402	C504	50-64	65+	1.000
14	2022-12		16	2022	0131863	L01CD01	PACITAXEL KABI	6MG/ML INF CNC SOL 1X50ML	6MG/ML	20.8092	MG	402	402	C504	50-64	65+	0.500
14	2023-01		17	2023	0131863	L01CD01	PACITAXEL KABI	6MG/ML INF CNC SOL 1X50ML	6MG/ML	20.8092	MG	402	402	C504	50-64	65+	0.500
14	2023-01		18	2023	0131863	L01CD01	PACITAXEL KABI	6MG/ML INF CNC SOL 1X50ML	6MG/ML	20.8092	MG	402	402	C504	50-64	65+	0.500
14	2023-01		19	2023	0250271	L01FY01	PHESGO	600MG/600MG INJ SOL 1X10ML	600MG/600MG	1.0000		402	402	C504	50-64	65+	1.000
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14	2023-03		22	2023	0191694	L02BG03	ANAPREX	1MG TBL FLM 100	1MG	100.0000	MG		402	C504	50-64	65+	1.000
14	2023-04		23	2023	0250271	L01FY01	PHESGO	600MG/600MG INJ SOL 1X10ML	600MG/600MG	1.0000		402	402	C504	50-64	65+	1.000
14	2023-04		24	2023	0250271	L01FY01	PHESGO	600MG/600MG INJ SOL 1X10ML	600MG/600MG	1.0000		402	402	C504	50-64	65+	1.000
14	2023-05		25	2023	0106016	L02BG03	ANASTROZOL TEVA	1MG TBL FLM 30	1MG	30.0000	MG		402		50-64	65+	3.000
14	2023-05		26	2023	0250271	L01FY01	PHESGO	600MG/600MG INJ SOL 1X10ML	600MG/600MG	1.0000		402	402	C504	50-64	65+	1.000

RWE Dataset can easily consist of 300 variables with large detail on patient history

**387 lecba
odbornost_prevazujici_zadatele**

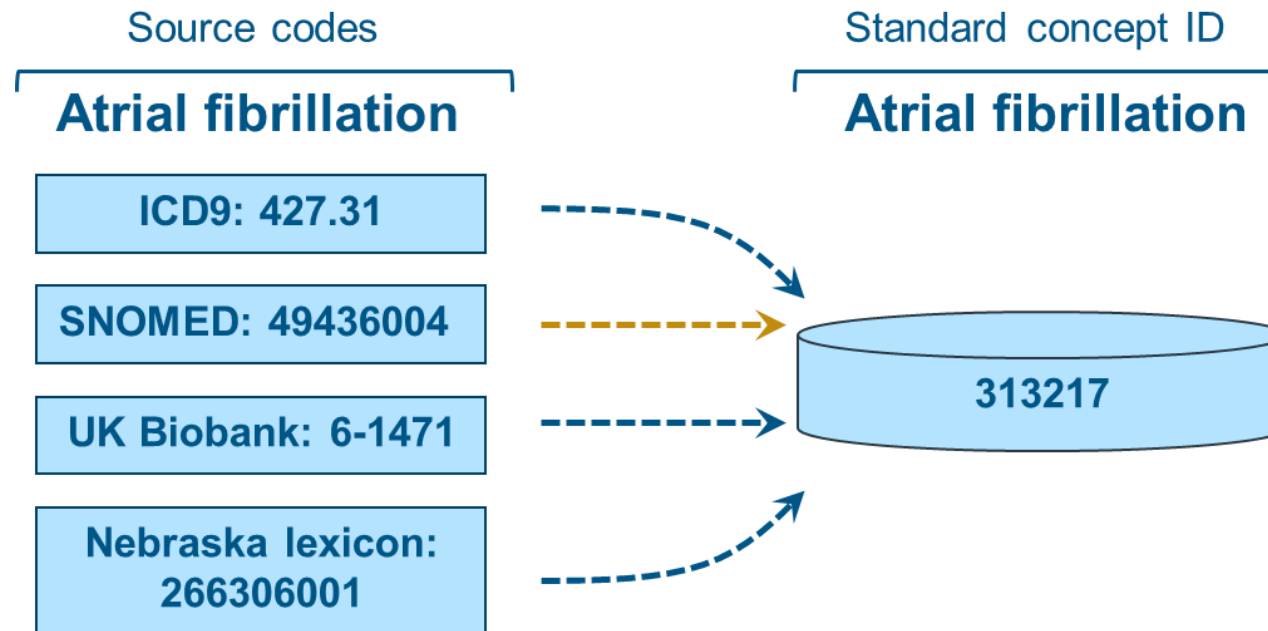
Registry of Newborns – Example of variables

- Type of intervention performance
- Reasons for ending the newborn report
- Position of the fetus in the uterus before birth
- Place where the birth occurred
- Causes of newborn death
- Hearing screening method: TEOAE or AABR
- Hearing screening result (normal or deviations detected)
- Method of administering vitamin K to newborns
- Method of newborn feeding after birth
- Modes of delivery
- Types of anesthesia used during childbirth
- Estimated date of birth based on personal identification number
- When diabetes was diagnosed in the mother
- Reasons for ending the report on the mother
- Type of twins (identical, fraternal) and placental arrangement
- Methods of conception
- Reasons for artificially induced labor
- Fetal position before cesarean section
- Person who assisted during childbirth
- Health status of the newborn at mother's discharge
- Course of cesarean section
- Course of spontaneous vaginal delivery
- Highest education level of the mother
- Evaluation of fetal cardiotocogram during pregnancy or labor
- Classification of newborn vitality
- Regions and districts according to NUTS
- Diagnoses according to the International Classification of Diseases
- Gender of the newborn
- Affiliation (Czech Republic, other EU member state, or non-EU country)
- Marital status of the mother
- Country code designation
- Admission date and time
- Education
- Marital status
- Total number of births
- Of which cesarean sections
- Nicotine use
- Drug use
- Body height
- Body weight at beginning of pregnancy
- Body weight at end of pregnancy
- 1st trimester combined screening (11-13 weeks)
- 2nd trimester detailed fetal morphology assessment (20-22 weeks)
- GBS screening result
- Pregnancy
- Placental disorders (multi-select)
- Serious pregnancy and birth complications
- Medications during pregnancy
- Other medication during pregnancy (note)
- Fetal position at start of labor
- Date of amniotic fluid rupture
- Start of 1st stage of labor (date+time)
- Start of 2nd stage of labor (date+time)
- Fetal birth (date+time)
- Placenta birth (date+time)
- Multiple pregnancy
- Type of multiple pregnancy
- Intrauterine fetal death
- Gestational age - weeks
- Gestational age - days
- Preinduction/induction of vaginal birth
- Method of preinduction/induction of vaginal birth
- Vaginal delivery completed
- Cesarean section completed
- Anesthesia during cesarean section
- Analgesia during vaginal birth
- Complications during birth
- Birth injuries (multi-select)
- Medications during birth
- Companion during birth
- Assistance during birth
- Postpartum complications
- Other postpartum complication - diagnosis
- Postpartum treatment
- Date of registry termination
- Reason for registry termination
- Child (fetus/newborn data)
- Order
- Gender
- Vitality
- Birth weight (g)
- Apgar score at 1 minute
- Apgar score at 5 minutes
- Apgar score at 10 minutes
- Arterial pH
- Lactate

OMOP Standardized vocabulary

Standardized coding system for clinical events

- Codes from other vocabularies have been mapped into OMOP concept IDs.
- Standard concept ID represents that clinical event throughout the OMOP CDM.



CZ claims data enables to track anonymized patient from birth, through diagnosis, treatment with great detail on oncology...

Illustrative patient journey (1/2)



Martina007, born on 1.1.1985, university graduate, married, Czech citizen, resident in the Prague district. Typically picks up medication at pharmacy X.

Meet Martina007

As a diabetic patient, Martina has been under the continuous care of Dr. Novak (diabetologist) since 2011. In the first years of the diabetes treatment, Martina did not take the prescribed medication X regularly. In 2015, Martina switched diabetologists, and her new doctor, Dr. Svoboda, prescribed a different treatment Y, which Martina adhered to. The physician perform test A&B and received payment of XYZ.

**Diabetes treatment
& HCPs switch**



Martina was admitted to the University Hospital Bulovka on 1.1.2015 at the age of 30, after an amniotic fluid leak earlier in the day. The delivery lasted 5 hours and took place 9 days before the planned delivery date. During the delivery, Martina was given treatment Z. No severe complications occurred, despite reporting 17 cigarettes per day. Bonding right after delivery. Martina's child was born 3kg and 55cm with an "Apgar score at 1 minute" of 8. after 5 min 10. PH 739. This was Martina's first delivery.

Childbirth



After a check-up at the gynecologist (Dr. Prochazka) on 1.1.2023, Martina was referred to the Oncology Clinic at General University Hospital in Prague. On 1.4.2023, Martina was diagnosed with a benign breast cancer with the following TNM system classifications: 2b, Xc, 1c. First tumor. Martina was prescribed treatment W. After one month, disease progression was observed. Morfology 84106.

**Referral to
oncology center**



... supported with unique info on transportation, vouchers, hospitalizations and death, all interpreted in volume as CZK value

Illustrative patient journey (2/2)

Martina also broke her left leg and was transported to Hospital X by emergency service at a cost of XYZ CZK.

After treatment, she was sent to health resort for rehabilitation at a cost of X CZK.

She also received a voucher for glasses and orthopedic equipment.

Two months later, Martina was hospitalized in Motol due to a reason XYZ. After 5 days she was discharged. The reason of discharge was “discharged to outpatient care”.

On Sunday 1.1.2048 our patient dies due to stroke with secondary reason being head injury.

In total she utilized insurance worth of 10m CZK





Real World Data Case studies

Client wanted to map patient journey prior to diagnosis of interest to identify potential bottlenecks of access to treatment

Patient journey prior to diagnosis

ILLUSTRATIVE

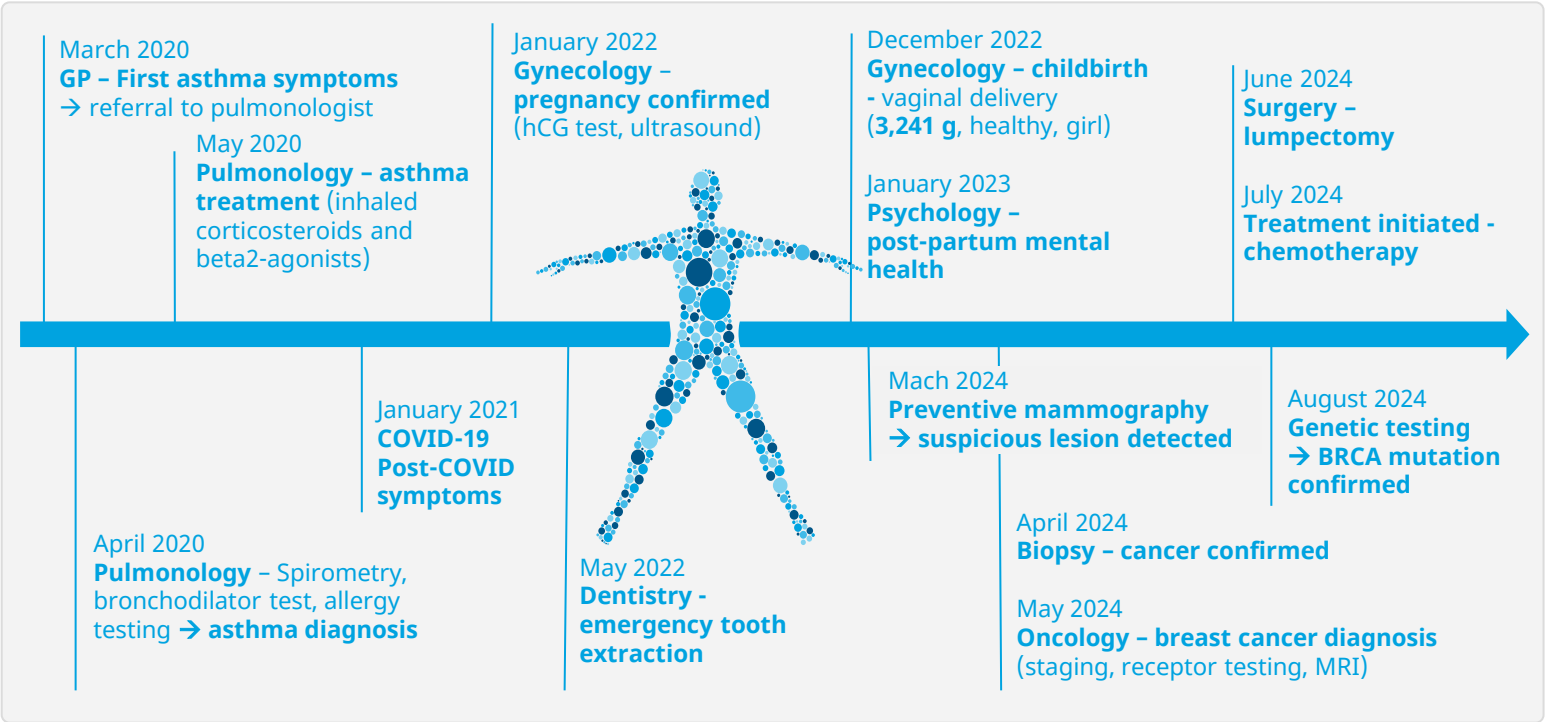
Situation

- Client hypothesized that less patients are being diagnosed with diagnosis of interest in CZ than can be assumed based on global prevalence data
- Client therefore asked IQVIA to map patient journey prior to diagnosis of interest and **identify potential bottlenecks in the patient journey** in CZ

Solution

- Using patient level **payer claims data**, IQVIA analyzed patient journeys with diagnosis of interest up to 2 years before the treatment start
- Patterns in other diagnoses, medication, specialties visited, and tests and procedures were used to create map of patient journey incl. share of drop offs
- Time spent in primary care before the patient was sent to specialist was also analyzed

Illustrative visualization – Individual patient journey



Client was launching a product in a new TA – advanced persistency curves were requested to see market dynamics

Persistence

ILLUSTRATIVE

Situation

- Client wanted to understand the market dynamics as well as competitor product usage
- With multiple indications, new launches and upcoming LoEs the market was very dynamic
- Several molecules on the market with varying dosing schemes

Solution

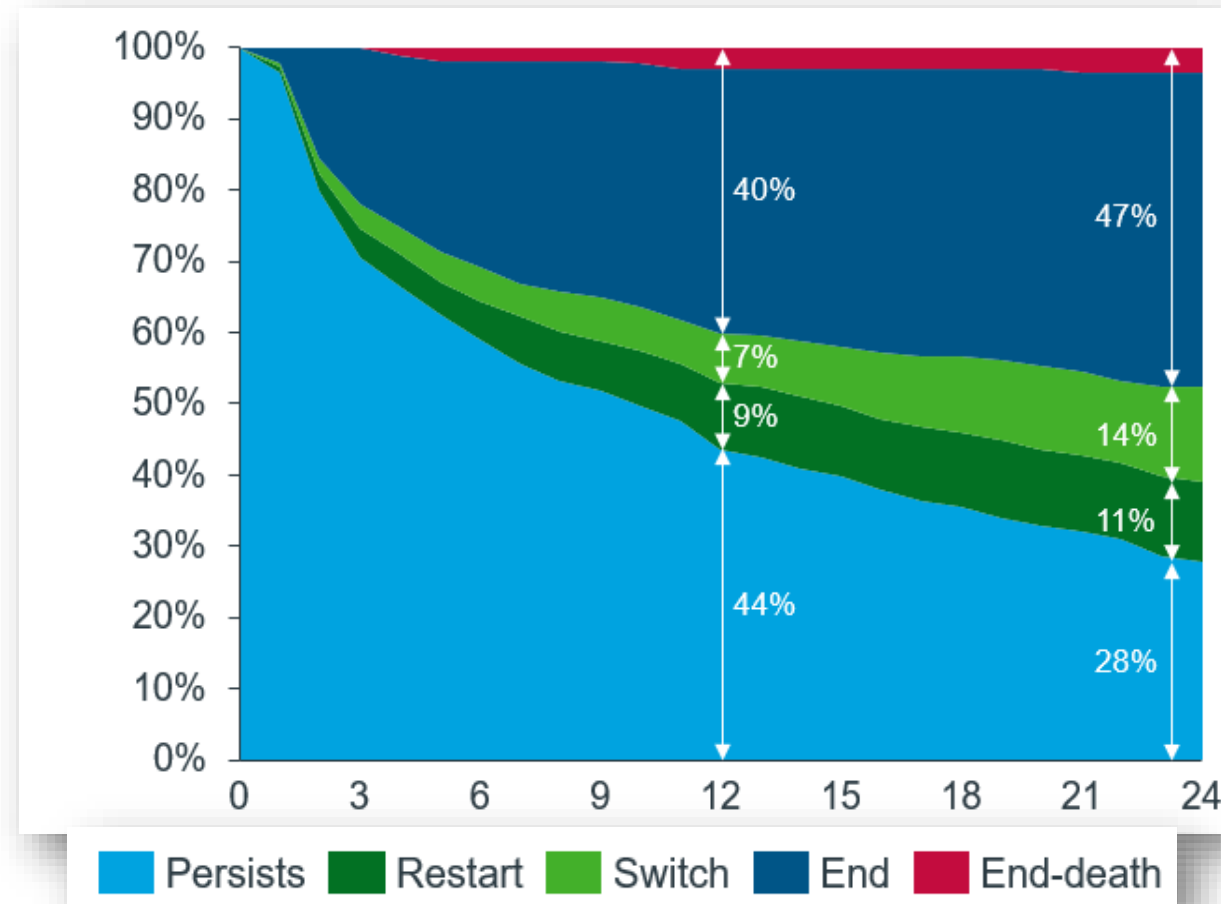
- Using patient level claims data, for given indication and patient cohorts, we have analyzed e.g. persistency
- Differences in KPIs across products were apparent
- When not persistent, patients switch, restart or end treatment

LoE: loss of exclusivity

Source: IQVIA

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Illustrative visualization – patient loss reason



Client wanted to register a new Cardiology FDC in Europe – IQVIA has provided RWE data on current co-prescription dynamics

Co-prescription data for drug registration

ILLUSTRATIVE

Situation

- Client wanted to register a new Fix-Dose-Molecule (FDC) combination within EU
- Based on the EMA safety requirements, client needs to bring RWE data from various counties on the number of patients that already are on the 2 existing mono-components

Solution

- IQVIA has collected secondary RWE data across EE countries, designedd a joint methodology, statistical plan and provided results in excel dashboard as well as a PDF report acceptable by national drug authorities

Illustrative visualization – co-prescription

	Patient count (projected)		
	MAT 06/2022	MAT 06/2023	MAT 06/2024
Molecule A (mono)	14,229	14,343	16,853
Molecule B (mono)	107,646	126,807	136,698
Molecule A (any)	21,020	25,098	26,052
Molecule B (any)	114,437	137,562	145,897
Molecule A + Molecule B (co-prescription)	6,791	10,755	9,199

The background is a dark blue field filled with glowing, wavy lines and numerous small, bright blue dots, creating a sense of digital motion and data flow. A large, white, rounded rectangular box is centered on the page, containing the hiring information.

We are hiring!
contact martin.cerny@iqvia.com

Thank you!

Martin Černý

Principal, CEE & CIS Real World Evidence

Martin.Cerny@iqvia.com

Barbora Jakubová

Consultant, CZ&SK Consulting & RWE

Barbora.Jakubova2@iqvia.com

